

BACKGROUND

- Ocrevus Zunovo® (ocrelizumab/hyaluronidase), is a recently FDA-approved combination of ocrelizumab, a widely used B-cell depleting monoclonal antibody used as a disease-modifying therapy for treatment of multiple sclerosis (MS), and hyaluronidase, an enzyme that increases the absorption of drugs into tissue.
- Ocrevus Zunovo® (ocrelizumab/hyaluronidase) is a 10-minute subcutaneous injection that can be administered in the clinic as an alternative to intravenous administration in an infusion center, allowing for increased patient convenience, decreased visit times, and reduced infusion center capacity constraints.

OBJECTIVE

Evaluate patient-reported experiences with ocrelizumab/hyaluronidase, including treatment-related side effects and pain or discomfort at three timepoints in patients receiving ocrelizumab/hyaluronidase subcutaneous injection in a real-world clinical setting.

METHODS

A standardized in-clinic administration process for subcutaneous ocrelizumab/hyaluronidase was implemented to evaluate patient experience and treatment tolerability. A total of 88 patients at a single center received treatment through this protocol.

The decision to start ocrelizumab/hyaluronidase was made through a **shared decision making process** during the patient's MS clinic visit and received education regarding the subcutaneous injection process, expected outcomes, and potential side effects. Pre-injection assessments included baseline patient-reported comfort level, anxiety, treatment expectations, and prior experience with their MS therapies.

Three time points were evaluated: (1) Pre-injection, (2) immediately post-injection, and (3) 72-hr post injection

Patients rated their (1) pain/discomfort and (2) injection-site reactions including visible side effects, such as erythema, swelling, or bruising.

The 72-hr post-injection assessments were conducted via electronic survey or via telephone to capture ongoing or delayed side effects, changes in discomfort, and patient perception of treatment tolerability. The multi-timepoint approach was designed to identify discrepancies between observed.

RESULTS

Characteristic		n = 88
Sex, n (%)	Female	51 (58)
	Male	37 (42)
Age (years)	Mean (SD)	44
	Range	21 - 71
Weight (kg)	Average	89.5
	Range	43.3 – 157.2
Race/Ethnicity, n (%)	White	65 (73)
	Black or African American	12 (14)
	Hispanic/Latino	8 (9)
	Middle Eastern/North African	3 (4)
Prior Treatment, n (%)	Ocrelizumab IV	51 (57.9)
	No prior treatment	14 (15.9)
	Fumarates	8 (9.1)
	S1P receptor modulators	4 (4.5)
	B-cell Depleting (non-Ocrelizumab)	4 (4.5)
	Glatiramer acetate	2 (2.2)
	Cladribine	2 (2.2)
	Natalizumab	1 (1.2)
	Teriflunomide	1 (1.2)
	Interferons	1 (1.2)

Table 1. Patient Demographics

RESULTS

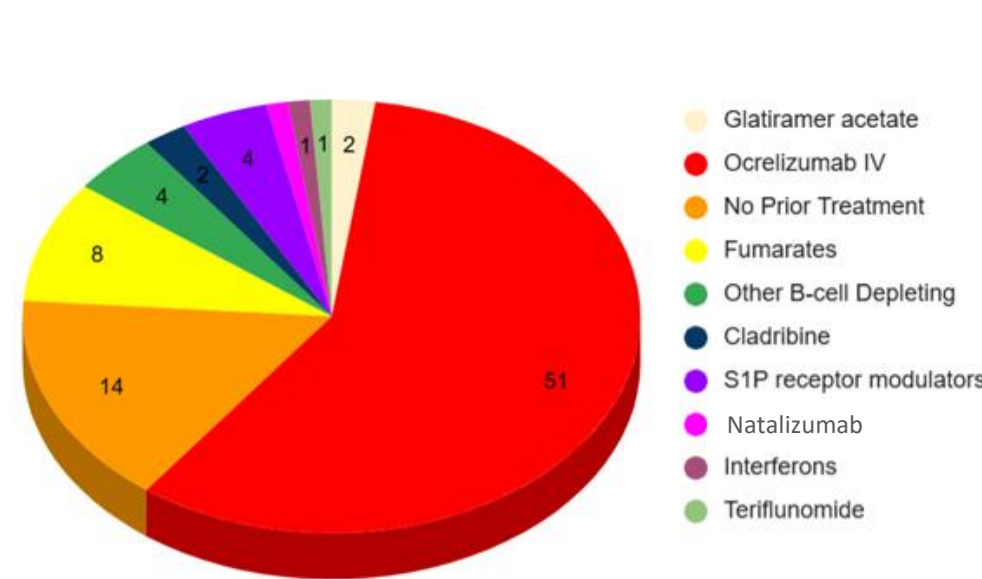


Figure 1. Previous Treatment

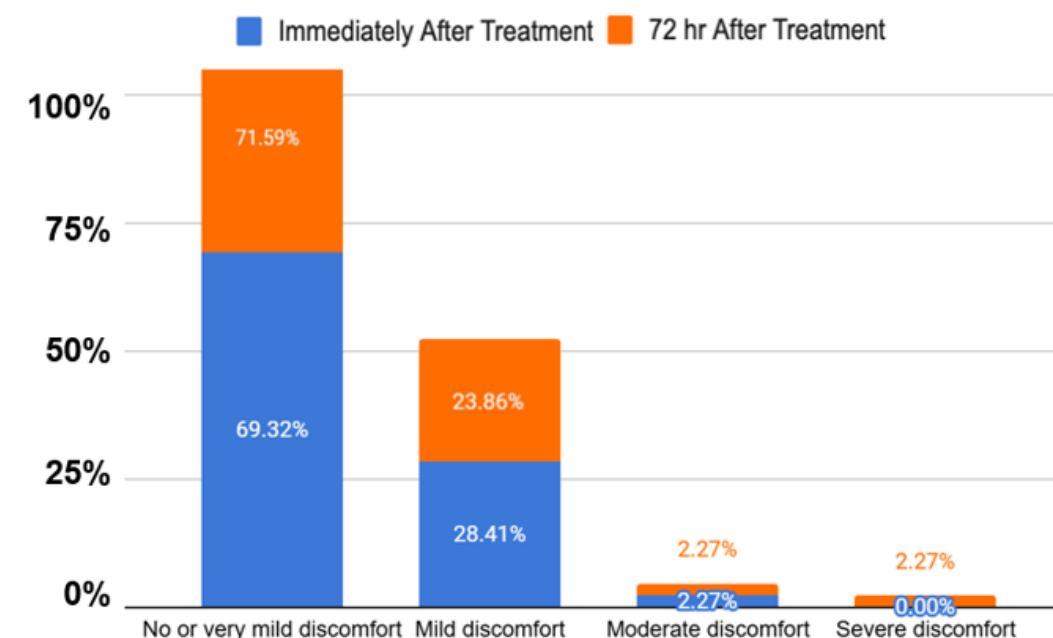


Figure 2. Patient reported discomfort/pain immediately after injection and 72 hrs after injection

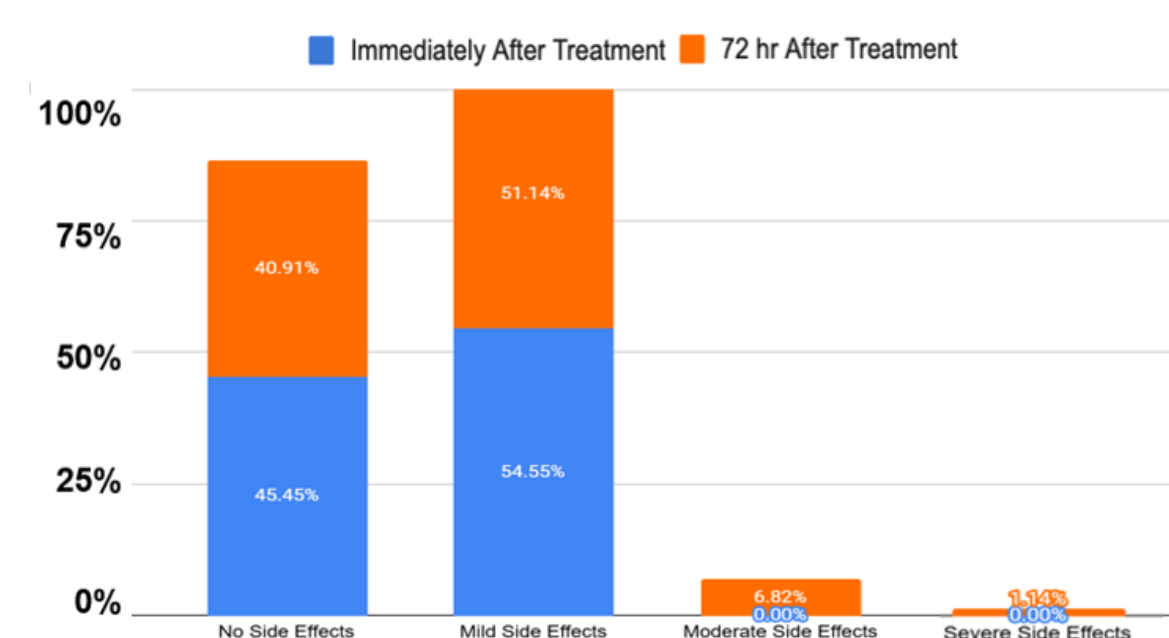


Figure 3. Patient reported side effects (ex. erythema/swelling) immediately after injection and 72 hrs after injection



Figure 4. No discomfort per patient report immediately post injection



Figure 5. Mild discomfort per patient report immediately post injection



Figure 6. Moderate discomfort per patient report 72-hrs post injection

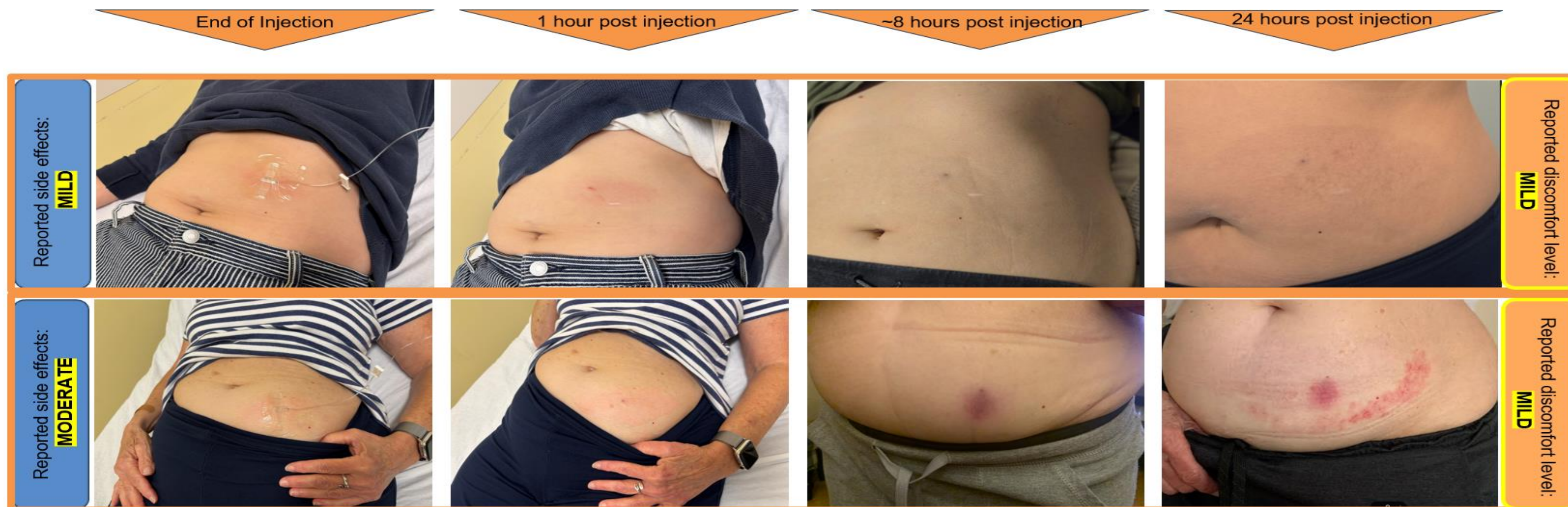


Figure 7. Progression of injection site reaction per patient report: (a) mild reaction vs (b) moderate reaction. Both patients reported mild discomfort

RESULTS



Figure 8. (a) Immediately post injection (b) Twenty minutes post injection

Example of mild side effects (erythema, swelling) immediately after completion of the injection and then 20 minutes later.

DISCUSSION

Evaluation of patient experience following in-clinic administration of subcutaneous ocrelizumab/hyaluronidase were highly positive and revealed that visual injection-site reactions did not consistently correlate with patient-reported discomfort. While patients reported very mild or mild findings such as erythema, swelling, or bruising, often this correlated with no to mild discomfort or pain. The majority of patients described minimal pain and high treatment tolerability throughout the process. These findings suggest that patient-perceived discomfort is generally low with subcutaneously administered ocrelizumab/hyaluronidase, and that even when visual side effects are reported, they rarely translate into significant symptoms of discomfort. Incorporating these patient-reported experiences provides valuable insight into real-world tolerability and can support counseling and expectation-setting for individuals initiating subcutaneous therapy.

CONCLUSIONS

- Ocrelizumab/hyaluronidase subcutaneous injection was well-tolerated, with the great majority of patients reporting no or only mild injection-site reactions (e.g., redness, swelling, itching, pain, tenderness, or burning).
- Real-world experience in our clinical setting confirmed the high patient satisfaction with ocrelizumab/hyaluronidase subcutaneous injection reported in OCARINA II.
- Clinic-administered subcutaneous ocrelizumab/hyaluronidase provides a convenient, efficient treatment option for MS patients.
- Real-world findings support the feasibility, safety, and patient-centered benefits of integrating ocrelizumab/hyaluronidase subcutaneous therapy into a routine neurology clinic workflow.

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